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June 16, 2025

Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013

Subject: Request for Information: Health Technology Ecosystem (CMS-0042-NC)

Dear Dr. Thomas Keane and Dr. Mehmet Oz,

Thank you for the opportunity to comment on the Request for Information (RFI): Health Technology Ecosystem. My comments are based upon my time as an employee for the Centers for Medicare & Medicaid Services (CMS) (retired in May, 2025)[i], my being a patient, and most importantly, one of four siblings currently helping our elderly parents make choices with regard to their health care.

General Comments

In the Summary, the RFI conveys the importance of empowering patients to make better decisions for their health and well-being and that CMS seeks input regarding the market of digital health products for Medicare beneficiaries as well as the state of data interoperability and broader health technology infrastructure. Although these are certainly worthy topics, none address the issues that are of most importance to Medicare beneficiaries or seek to resolve the problems that beneficiaries and their representatives[1] face every day.

The most glaring absence in the RFI is the lack of any reference to improper payments or fraud, waste, and abuse. Using human-centered design to prioritize the needs, preferences, and limitations of beneficiaries could facilitate the building of a digital healthcare system that simplifies messaging to beneficiaries, unifies providers and developers, and promotes innovation. This approach would also facilitate reductions in improper payments and fraud, waste, and abuse while also recognizing beneficiaries have a role beyond being forced to use whatever system is chosen for them.

1. Beneficiaries must be given accurate and immediate information regarding the coverage of individual items and services, as well as their financial responsibilities. Medicare coverage requirements continue to be difficult for both beneficiaries and providers to identify. Even when identifiable, determining the actual meaning of the requirements can be problematic with the same language being interpreted differently by different contractors. Some coverage requirements, such as those for wound care, can be traced to retired local coverage determinations. Add to this, conditions of participation, conditions of payment, documentation requirements, etc., and both providers and beneficiaries struggle to understand if or when items and services are covered.

[1] The RFI uses various terms, e.g., "loved ones," "caregiver," "representatives," etc. For consistency, this response will use the term representative except where language is quoted directly from the RFI.

The confusion surrounding the identification of all coverage requirements is propagated as a way of impeding even more fraud, waste, and abuse because making it more difficult to identify requirements will make it difficult for "bad actors" to figure out if or when Medicare covers specific items and services. This approach has not proven accurate. "Bad actors" have no problem discovering new ways to commit fraud, but providers and beneficiaries remain unable to determine whether items and services are reasonable and necessary. It is time for CMS to change its approach and clearly map the requirements for every item and service within any new technology, so this information is available in real time. Not only will this approach assist beneficiaries, but it will also reduce the burden on providers.

2. Notifications of changes in Part A coverage are equally important in stopping fraud, waste, and abuse. For example, hospice-related fraud is significant, and when a patient is fraudulently enrolled in hospice, the ability to receive other Medicare benefits stops. Currently, a patient may not find out about the change until they need non-hospice-related care and are denied. If properly addressed, new technology must notify the beneficiary in real time of changes in coverage, e.g., a change to hospice, so that the patient (or their representative) can immediately address an erroneous or fraudulent change with CMS. This model must be replicated for all similar changes.

3. New technology should also result in additional changes that will help beneficiaries and reduce the burden on providers. As an example, rather than CMS relying on a provider to give a Medicare beneficiary an advance beneficiary notice (ABN) (CMS-R-131), the beneficiary could receive the ABN electronically through an app while in their provider's office. The beneficiary would immediately know the reason Medicare has deemed an item or service not reasonable and necessary under §1862 [42 U.S.C. 1395y] along with the cost of the item or service. Additionally, a beneficiary that receives an ABN when not actually at a provider's office would have some knowledge that potentially fraudulent events are occurring.

   In the same way, the new technology should also be used to convey other notices, including discharge appeal notices, so that beneficiaries and their family have immediate information about appeal rights. And once again, by incorporating these functions into an app, the burden placed on providers would be reduced while also giving CMS assurance that the beneficiary received accurate and thorough information when it was needed.

4. Beneficiary health and well-being and the market of digital health products need to begin with protecting beneficiaries from the constant attempts to gain access to their information and their financial resources. This is of vital importance to beneficiaries, most of whom are on fixed incomes. It is also the main concern of the representatives helping them. Patients, including my parents and myself, are inundated with phone calls, texts, e-mails, and regular mail from "bad actors" attempting to steal personal health information and money. The "bad actors" will not stop trying, and the health technology infrastructure must be designed with these situations in mind.

   Rather than engaging with various phone calls, text messages, etc., a beneficiary must be given a method to immediately check a single app or website to determine the validity of any contact from CMS or their health care providers. This technology is already available through financial institutions, including banks and investment companies, and given the financial ramifications, it should be used for an individual's health care. To be effective, the information must be conveyed in real time so that patients - as the first line of defense in fighting fraud, waste, and abuse and improper payments - can easily distinguish legitimate requests from illegitimate attempts to steal their information or money.

5. The app must notify beneficiaries of current "phishing" and fraud-related schemes. This includes notifications tailored to the beneficiary should the beneficiary be the recipient of an item or service that CMS has marked as having a higher probability of fraud. The notification can also advise the beneficiary or representative what action should be taken in response to receiving erroneous or fraudulent items or services.

6. For some time, the CMS, Center for Program Integrity (CPI) has considered the possibility of conducting a "Chili Cook-Off," i.e., a competition amongst artificial intelligence (AI) and information technology (IT) companies to develop a system to carry out automated medical record reviews. Such a system would greatly benefit CMS' efforts to combat improper payments and fraud, waste, and abuse given that CMS processes over one billion claims annually, but by some estimates reviews less than .01% of all claims. Through the "Chili Cook-Off," each company would be given the opportunity to build and then demonstrate their solution. Unfortunately, CPI has met resistance in carrying out the "Chili Cook-Off" because of the decision that there is no statutory authority to conduct such a competition. This decision seems counter to the direction of the current administration. The Department of Health and Human Services must re-look at this issue and make a clear decision as to whether such authority does not exist and work with the current administration to either develop a work-around or push for immediate Congressional action.

7. When attempting to procure new AI or IT systems, including those to conduct medical record review, CMS has relied upon one contract, an existing IDIQ, rather than establishing a new contract specific to each need. This approach has limited the ability to explore new companies that may have ground-breaking technology. Taking an alternative approach where each contractor focuses upon the functional needs will facilitate procuring the technology actually sought.

8. CPI is also limited in the type of demonstrations conducted. Section 402(a)(1)(J) of the Social Security Amendments of 1967, as amended [42 U.S.C. 1395b-1(a)(1)(J)] authorizes the Secretary to "develop or demonstrate improved methods for the investigation and prosecution of fraud in the provision of care or services under the health programs established by the Social Security Act [the Act]." CPI has used this authority on numerous occasions, but the plain language limits these demonstrations to those related to the "investigation and prosecution of fraud." CPI needs broader authority to conduct demonstrations that are not only focused on fraud, but also on waste, abuse, and improper payments. If deemed necessary, CMS must pursue immediate Congressional intervention to enable demonstrations with a broader purpose.

9. With the establishment of the Fraud Prevention System (FPS), CMS began moving away from "pay and chase." However, upon identifying potential fraud through the FPS, CMS does not make any effort to determine whether CMS' coverage requirements and policies should be changed or refined to eliminate those aspects being exploited by "bad actors." Without this, the cycle of "bad actors" committing the same fraud will continue, the FPS will continue identifying the fraud, and new "bad actors. CMS is not decreasing opportunities for fraud. To eliminate this cycle, a direct line of communication must be established between CPI and CM so discussions can occur about whether specific coverage requirements or policies should be changed.

Responses to Specific Questions

PC-1. What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?

RESPONSE:

1. Any app must provide immediate information to the beneficiary (or his representative/family members) about the coverage of items or services that providers are ordering. This includes information on whether prior authorization is needed BEFORE leaving the provider's office since too often, patients leave only to find out later that prior authorization was needed. This also includes the electronic issuance of an ABN or other notice that includes the cost of the item or service so that a beneficiary is not surprised when receiving a bill at a much later date. Again, the coverage requirements should be clear and easy to understand so that the decisions can be made in real time and the negative impact on beneficiaries minimized.
2. The app should enable multiple users to access the app (as approved by the beneficiary). Many times, multiple family members are involved in the care of a parent, including transporting them to healthcare appointments, carrying out medical treatments, or ensuring medications are taken as scheduled.
3. Visit-summaries must be easily accessible. Frequently, patients are overwhelmed by all the information conveyed during an appointment. Making summaries immediately available through an app would help those beneficiaries or their representatives

PC-1. a. What are the top things you would like to be able to do for your or your loved ones' health that can be enabled by digital health products?

RESPONSE: Rather than relying on providers or third-party vendors to forward health records, give beneficiaries (and loved ones) total access to their own health records so that they can send or provide access to their records. Thus, the only individual that could block the movement of a beneficiary's medical information is the beneficiary.

PC-2. Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)?

RESPONSE: No.

PC-2. b. If not, in what contexts or for what workflows would it be most valuable to use one portal or system to access all such health information?

RESPONSE: As previously noted, all health information must be accessible with the beneficiary or representative having the authority to send their health information to another provider.

PC-4. a. What apps should exist but do not yet? Why do you believe they do not exist yet?

RESPONSE: Currently, patients cannot view their entire medical record, including x-rays, lab results, visit summaries, from all their providers through a single app. Nor can patients use a single app to schedule appointments with their different providers. This type of care-coordination must be simplified for beneficiaries and their representatives.

PC-5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

RESPONSE: Rather than hoping that patients and their representatives use the technology, the technology should be part of the initial entry into Medicare. While it is true that some beneficiaries cannot use this technology and might decline the offer, a new era of beneficiaries and representatives are arriving. In the near future, most beneficiaries and their representatives will already be totally immersed in technology and realize the usefulness of an "all in one place" app for their healthcare needs.

PC-5. a. What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality, or impact or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process? What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of

digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)?

RESPONSE: CMS is the entity who can best determine the standards that vendors must meet when developing apps. CMS can then notify all vendors of these standards, which must be met to even be considered for CMS certification.

PC-13. How can CMS encourage patients and caregivers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would increasing reporting of complaints advance or negatively impact data exchange?

RESPONSE: The easiest way to resolve this is to give beneficiaries total access and control over their data. Rather than a provider making the decision on whether to release data, the beneficiaries could enter the app and pick specific providers with whom they want their healthcare information shared. This control would include the ability to take away a provider's access.

PR-1. b. What information should providers share with patients when using digital products in the provision of their care?

RESPONSE: As previously addressed, the question should not be what information a provider shares with patients but what information should patients share with their providers. Note that the default should certainly be that all information is shared with a beneficiary's specific provider, but the beneficiary should have the control.

PR-1. c. What responsibilities do providers have when recommending use of a digital product by a patient?

RESPONSE: Currently, providers have total authority to choose what technology they use. Beneficiaries have no choice but to use the technology the provider selects, i.e., it is not a recommendation. The only way to avoid using the technology is by calling the provider every time you need an appointment or prescription or by changing providers.

Respectfully,

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[i] Thomas J. Kessler was most recently an employee of the CMS, Center for Program Integrity, Provider Compliance Group where he wrote several regulations, including the Hospital Outpatient Department Prior Authorization Process regulations; and led efforts to use information technology and artificial intelligence in conducting medical record review and prior authorization. Thomas previously identified improvements to CMS' payment suspension process and facilitated the completion of the Fraud Prevention System Year 1 and Year 2 Reports to Congress. He also led the Center for Clinical Standards & Quality, Quality Improvement Organization's (QIO) 9th Scope of Work Beneficiary Protection Program theme, which includes all statutorily required reviews carried out by the QIOs, revised the QIO regulations, including the data accessibility regulations, and led the QIO's transition from its prior state-based structure. Previously, he served as CMS' limitation on liability (§1879/§1862) subject matter expert and assisted in the negotiations of the hospital discharge appeals process while in the Center for Medicare's, Medicare Enrollment & Appeals Group.